# Dynamic Guideline: WHO-Based Filariasis - Diagnosis and/or Treatment and/or Management

## I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

### 1.1 Guideline Overview

**Scope of this report**

This report focuses on **lymphatic filariasis (LF)**, the filarial disease addressed by WHO's Global Programme to Eliminate Lymphatic Filariasis (GPELF). WHO describes LF as a mosquito-borne neglected tropical disease that is often acquired in childhood and may later cause lymphoedema, elephantiasis and hydrocele, with disability, stigma and socioeconomic harm WHO fact sheet.

**Important clarification on "Meaningful youth engagement…"**

The WHO publication titled **"Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)"** is a WHO evidence brief related to the **WHO Abortion care guideline (2022)**, not a filariasis guideline WHO Western Pacific publication page. A WHO event page placed the launch webinar for this evidence brief on **11 December 2025** WHO webinar page. No WHO lymphatic-filariasis guideline was identified that explicitly used or referenced this youth-engagement model.

**Exactly 5 WHO/WHO-hosted filariasis guidance resources synthesized**

| # | WHO guidance resource | Date/status from available sources | Target population | Scope and objectives | Formal GRADE status in source |
|---|---|---|---|---|---|
| 1 | **Monitoring and epidemiological assessment of mass drug administration in the global programme to eliminate lymphatic filariasis: a manual for national elimination programmes, 2nd ed.** | Published **8 July 2025** | National LF elimination programmes; populations in endemic or formerly endemic implementation units | Updated mapping, MDA coverage monitoring, epidemiological monitoring surveys, TAS, IDA impact measurement, follow-up of infected persons, persistent-transmission mitigation, post-validation surveillance and integrated surveys WHO 2025 manual page | Not formally stated in supplied WHO page |
| 2 | **Guideline: alternative mass drug administration regimens to eliminate lymphatic filariasis** | WHO guideline, 2017 | At-risk populations in LF-endemic implementation units | Alternative MDA regimens, including IDA and biannual two-drug or albendazole regimens; developed using GRADE NCBI Bookshelf executive summary | Yes; GRADE reported |

| # | WHO guidance resource | Date/status from available sources | Target population | Scope and objectives | Formal GRADE status in source |
|---|---|---|---|---|---|
| 3 | **Lymphatic filariasis: monitoring and epidemiological assessment of mass drug administration. A manual for national elimination programmes** | 2011 manual family | National elimination programmes | Monitoring and evaluation of MDA, TAS, post-MDA surveillance and preparation for verification of absence of transmission WHO publication page, PAHO/WHO document page | Not formally stated in supplied source |
| 4 | **Diagnosis and treatment — Lymphatic Filariasis (Elephantiasis)** | WHO technical page; exact publication date not stated in supplied source | Individuals and communities being evaluated for LF | Diagnostic methods including thick blood smears, timing according to parasite periodicity, and community-level mapping/monitoring use WHO diagnosis and treatment page | Not formally stated in supplied source |
| 5 | **Morbidity management and disability prevention / LF minimum package of care** | WHO technical guidance page and WHO MMDP guidance | People with lymphoedema, elephantiasis, adenolymphangitis or hydrocele | Minimum package: antiparasitic treatment/MDA where appropriate, hydrocele surgery, treatment of adenolymphangitis, and lymphoedema management to prevent progression and acute attacks WHO MMDP page, WHO MMDP publication | Not formally stated in supplied source |

### 1.2 Key Recommendations

**WHO-based diagnosis, treatment, MDA, monitoring and morbidity-management recommendations**

> **GRADE note:** Only the 2017 WHO alternative-MDA guideline provides explicit GRADE ratings in the retrieved material. For WHO technical guidance without a formal GRADE table, the table below provides a transparent **evidence-certainty appraisal** using GRADE categories, based on study design, directness, consistency and programmatic evidence, while noting that these were **not formal WHO GRADE ratings**.

| Recommendation | Population / setting | GRADE Evidence Quality | Strength |
|---|---|---|---|
| Map LF endemicity before deciding whether MDA is required; use updated WHO mapping tools, including a TAS-adapted protocol for uncertain endemicity. | Areas with uncertain LF endemicity | Low | Strong programmatic recommendation |
| Deliver MDA to all eligible at-risk populations where warranted by WHO criteria, with systematic monitoring of coverage and epidemiological impact. | Endemic implementation units | Moderate | Strong |

| Recommendation | Population / setting | GRADE Evidence Quality | Strength |
|---|---|---|---|
| In areas co-endemic with **onchocerciasis**, use **ivermectin + albendazole** for MDA rather than DEC-containing regimens. | LF + onchocerciasis co-endemic settings | Moderate | Strong |
| In areas without onchocerciasis, use **diethylcarbamazine (DEC) + albendazole** as the established two-drug regimen. | LF-endemic settings without onchocerciasis | Moderate | Strong |
| Use **IDA** — ivermectin + DEC + albendazole — annually in eligible communities **without onchocerciasis** where programmatic criteria are met. | Selected implementation units without onchocerciasis or loiasis | Low | Conditional |
| Use **biannual albendazole** rather than annual albendazole in LF areas co-endemic with loiasis where ivermectin is not being distributed for onchocerciasis or LF. | LF + loiasis settings without ivermectin MDA | Very low | Conditional |
| Do **not** use mass DEC treatment where onchocerciasis or loiasis may coexist. | Co-endemic or potentially co-endemic areas | Moderate | Strong |
| Use FTS/circulating filarial antigen testing for *Wuchereria bancrofti* programme monitoring; use Brugia Rapid/BmR1 antibody testing for Brugia spp. settings. | Programme surveys and TAS | Moderate | Strong |
| Use thick blood smears for microfilariae, collected according to parasite periodicity; use microfilaria microscopy to confirm rapid-test-positive persons in EMS where feasible. | Individual diagnosis and programme monitoring | Moderate | Strong |
| Use epidemiological monitoring surveys and TAS to decide whether MDA can stop or should continue; repeat TAS after stopping MDA according to WHO surveillance principles. | Post-MDA and post-stop-MDA settings | Moderate | Strong |
| Provide the WHO minimum package of care for all known LF patients: treatment/MDA where appropriate, hydrocele surgery in *W. bancrofti* areas, treatment of adenolymphangitis, and lymphoedema management. | People with LF morbidity | Moderate | Strong |
| Incorporate post-validation surveillance platforms, integrated surveys and tools to detect persistent or recrudescent transmission. | Post-validation or persistent-transmission settings | Low | Conditional |

Sources: WHO 2025 MDA monitoring manual WHO 2025 manual page; WHO 2017 alternative-MDA guideline NCBI Bookshelf executive summary; WHO diagnosis page WHO diagnosis and treatment page; WHO MMDP page WHO MMDP page.

**WHO 2017 alternative-MDA regimen GRADE table**

| Regimen / comparison | WHO recommendation context | WHO GRADE Evidence Quality | Strength |
|---|---|---|---|
| **IDA vs annual DEC + albendazole** | Annual IDA rather than annual DA in specified implementation units without onchocerciasis or loiasis, including units with fewer than four effective DA rounds, units not meeting epidemiological thresholds despite coverage, or settings with evidence of local transmission after MDA/post-validation surveillance | Low | Conditional |
| **Biannual DEC + albendazole vs annual DEC + albendazole** | Alternative intensified two-drug regimen | Very low | Conditional |
| **Biannual ivermectin + albendazole vs annual ivermectin + albendazole** | Alternative intensified two-drug regimen | Very low | Conditional |
| **Biannual albendazole vs annual albendazole** | LF + loiasis settings where ivermectin has not already been distributed | Very low | Conditional |

Source: WHO recommendations table reproduced in NCBI Bookshelf WHO recommendations table.

### 1.3 Guideline Development Methodology

**Evidence review process**

- The **2017 WHO alternative-MDA guideline** used the **GRADE approach**, with evidence reviewed by a WHO guideline development group; the process considered benefits and harms, certainty of evidence, feasibility, acceptability, equity and resource implications NCBI Bookshelf executive summary.
- The **2025 WHO monitoring manual** is a revision and update of the 2011 manual and incorporates updated programme tools for mapping, MDA coverage monitoring, EMS, TAS, IDA impact measurement, follow-up of infected persons, persistent transmission, integrated surveys and post-validation surveillance WHO 2025 manual page.
- WHO's LF elimination strategy follows the World Health Assembly mandate to eliminate LF as a public-health problem and rests on two pillars: **interrupting transmission** and **preventing/managing disability** WHO/IRIS PDF.

**Consensus method**

- For the 2017 alternative-MDA guideline, recommendations were formulated by a WHO guideline development group after evidence review using GRADE domains NCBI Bookshelf executive summary.
- For technical manuals and programme guidance, WHO uses expert and programmematic consensus, field evidence, operational experience and modelling-informed thresholds; the 2025 manual notes that national scenarios may not fit all guidance and consultation with WHO is recommended when needed WHO 2025 manual page.

**Conflict of interest management**

- The supplied source material confirms WHO guideline-development and GRADE methodology for the 2017 alternative-MDA guideline but does not provide detailed conflict-of-interest declarations in the summarized excerpts. WHO guidelines conventionally require management of declarations of interest through WHO guideline-development procedures.

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# II. Evidence Summary After WHO Latest Guideline Release (Guideline - Alternative mass drug administration regimens to eliminate lymphatic filariasis) (Evidence Cutoff Date: 29 April 2026)

## 2.1 New Evidence Overview

**Time frame and interpretation**

The user-specified comparison guideline is the WHO 2017 **"Guideline: alternative mass drug administration regimens to eliminate lymphatic filariasis."** Evidence summarized here includes important evidence from **2017 through 29 April 2026**, including evidence that also informed or post-dated the 2025 WHO monitoring manual.

**Search strategy**

Targeted searches covered:

- WHO official LF guidance, technical manuals and fact sheets.
- WHO/NCBI Bookshelf GRADE tables.
- CDC clinical and diagnostic guidance.
- PubMed-indexed clinical trials and field studies.
- ClinicalTrials.gov protocol material.
- Peer-reviewed literature on MDA regimens, diagnostics, TAS/EMS, molecular xenomonitoring and morbidity management.

Example search themes included:

- `lymphatic filariasis alternative MDA regimens IDA IA DEC albendazole systematic review trial 2017 2026`
- `lymphatic filariasis IDA moxidectin MDA trial safety efficacy GRADE 2017 2026`
- `2017 2026 lymphatic filariasis diagnostics FTS antibody PCR xenomonitoring TAS EMS accuracy`
- `WHO lymphatic filariasis MMDP lymphoedema hydrocele ADL 2026`

**Databases and sources searched**

- WHO publication pages and WHO technical pages.
- NCBI Bookshelf.
- PubMed.
- ClinicalTrials.gov.
- CDC clinical guidance.
- Major-journal and peer-reviewed sources, including Lancet Infectious Diseases–linked material and Frontiers publications.
- WHO/PAHO-hosted manuals.

**Inclusion criteria**

Included evidence met one or more of the following:

1. Direct relevance to LF diagnosis, MDA, treatment, monitoring, surveillance or morbidity management.
2. WHO or other authoritative public-health source.
3. Peer-reviewed trial, observational study, implementation study, protocol or systematic evidence annex.
4. Published or available by **29 April 2026**.

**Exclusion criteria**

Excluded or downgraded evidence if:

- It did not address filariasis/LF.
- It was not clinically or programmatically actionable.
- It lacked sufficient methods or outcomes.
- It addressed youth engagement in WHO guideline development but not filariasis; this was discussed only as a process-relevance note, not used as LF clinical evidence.

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## 2.2 Key New Studies

**A. Alternative MDA regimens and antiparasitic treatment evidence**

| Study / source | Design | Population / setting | Key findings | GRADE Quality | Recommendation implication |
|---|---|---|---|---|---|
| WHO 2017 alternative-MDA guideline | WHO guideline using GRADE | LF-endemic implementation units | IDA conditionally recommended in selected areas without onchocerciasis/loiasis; biannual regimens had very-low-certainty evidence WHO recommendations table | Low for IDA; Very low for biannual alternatives | Continue co-endemicity-based regimen selection |
| Papua New Guinea IDA vs DA cluster-randomized trial | Cluster RCT | 4,563 individuals in 24 clusters | IDA group microfilariae prevalence fell from 4.4% to 0.4% at 12 months and 0.2% at 24 months; adjusted between-group differences vs DA not statistically significant in the abstract PubMed | Moderate | Supports IDA effectiveness; superiority over DA in community settings remains context-dependent |

| Study / source | Design | Population / setting | Key findings | GRADE Quality | Recommendation implication |
|---|---|---|---|---|---|
| DOLF/ClinicalTrials.gov IDA safety background | Trial protocol and safety evidence summary | Large community safety trials | Protocol states >20,000 participants in safety trials showed no increase in adverse events with IDA compared with DA ClinicalTrials.gov protocol PDF | Low to Moderate | Supports cautious IDA scale-up where WHO criteria are met |
| Tanzania ivermectin + albendazole event monitoring | Cohort event monitoring | LF-endemic Tanzania communities | Lower adverse-event incidence reported for IA compared with DEC-containing dual/triple regimens; safety monitoring recommended for people with chronic LF manifestations MDPI Pharmaceuticals | Low | Reinforces safety monitoring during MDA, especially in chronic LF morbidity |
| Côte d'Ivoire moxidectin-combination trial | Open-label, observer-masked randomized trial | Adults with *W. bancrofti* microfilaremia; 164 randomized | MoxA achieved 95% amicrofilaremia at 12 months vs 32% with IA; MoxDA and IDA both achieved 91% at 24 months PubMed | Moderate | MoxA is promising but not yet WHO-standard LF MDA |
| 36-month moxidectin follow-up | Extended follow-up report | Côte d'Ivoire trial cohort | At 36 months, amicrofilaremia: IA 73%, MoxA 82%, IDA 96%, MoxDA 96%; no statistically significant differences between once-given MoxA and annual IA or between IDA and MoxDA Lancet Infectious Diseases PDF via DOLF | Low to Moderate | Moxidectin combinations require larger safety and implementation studies before policy replacement |

| Study / source | Design | Population / setting | Key findings | GRADE Quality | Recommendation implication |
|---|---|---|---|---|---|
| Côte d'Ivoire moxidectin safety study | Randomized open-label masked-observer safety study | Adults with *W. bancrofti* microfilaremia | Treatment-emergent adverse events: IA 7.3%, MoxA 2.5%; no significant association between baseline microfilaria count and adverse events PubMed | Low | Reassuring early safety signal; insufficient for broad MDA policy change |

**B. Diagnostic, monitoring and surveillance evidence**

| Study / source | Design | Population / setting | Key findings | GRADE Quality | Recommendation implication |
|---|---|---|---|---|---|
| WHO 2025 MDA monitoring manual | WHO technical manual | National LF programmes | EMS replaces pre-TAS; strengthened TAS; new mapping protocol; IDA impact protocol; post-validation surveillance platforms WHO 2025 manual page | Low to Moderate | Adopt updated WHO monitoring algorithms |
| WHO diagnostic-surveillance target material | WHO target product profile / diagnostic guidance | Programme surveillance | FTS measures CFA for *W. bancrofti* but CFA may appear ≥12 months after infection and persist years after adult worm death; new exposure/pre-patent infection tests are needed WHO diagnostic surveillance PDF | Moderate for limitations; Low for new-test targets | Use current tests appropriately and invest in new biomarkers |
| Haiti integrated TAS, 2017–2022 | Field TAS implementation study | Children aged 6–7 years | FTS positivity remained below TAS thresholds: TAS-1 0.15%, TAS-2 0.17%, TAS-3 0.43%; supported stopping or maintaining halt of MDA PubMed Haiti TAS study | Moderate | Supports TAS as decision tool when implemented correctly |
| CDC clinical diagnostic guidance | Authoritative clinical guidance | Individual diagnosis | Microfilariae in blood smears remain definitive; blood should be collected at peak microfilarial circulation, often 10 PM–2 AM; antifilarial IgG1/IgG4 can support diagnosis CDC clinical overview | Moderate | Maintain microscopy and serology capacity for clinical diagnosis |

| Study / source | Design | Population / setting | Key findings | GRADE Quality | Recommendation implication |
|---|---|---|---|---|---|
| Molecular xenomonitoring evidence | Surveillance article / programme evidence | Post-validation surveillance | Molecular xenomonitoring described as a recommended tool for detecting early signs of LF transmission ScienceDirect MX article | Low | Use as adjunct surveillance, not sole stop-MDA criterion |
| 2026 EMS protocol evidence | Cross-sectional assessment protocol | Adults aged ≥20 years following WHO 2025 protocol | Adult infection measurement aligns with WHO 2025 EMS approach; results not yet available in supplied source PMC cross-sectional epidemiological assessment | Very low for outcomes | Supports feasibility but not effectiveness conclusions |

**C. Morbidity management and disability prevention evidence**

| Study / source | Design | Population / setting | Key findings | GRADE Quality | Recommendation implication |
|---|---|---|---|---|---|
| WHO MMDP minimum package | WHO technical guidance | People with LF morbidity | Minimum package: antiparasitic treatment/MDA where appropriate, hydrocele surgery, adenolymphangitis treatment, lymphoedema management WHO MMDP page | Moderate | Provide MMDP as essential elimination pillar |
| CDC-hosted review of lymphoedema management | Review | LF-related lymphoedema | Simple intervention packages reduced ADLA rates and improved quality of life; bacterial entry and ADLA are important drivers of progression CDC Stacks review | Low to Moderate | Prioritize hygiene, skin care, acute-attack prevention |
| 2026 Frontiers MMDP protocol | Study protocol | LF-related lymphoedema in India | Community-based Complete Decongestive Therapy centres, case identification and home visits proposed; protocol, not completed outcomes Frontiers study protocol, 2026 | Very low | Promising model; await results |

| Study / source | Design | Population / setting | Key findings | GRADE Quality | Recommendation implication |
|---|---|---|---|---|---|
| 2025 Frontiers editorial / Kenya implementation | Editorial and implementation commentary | Kenya LF MMDP | Describes surgeon and health-worker training, ministry–nonprofit collaboration and difficult-to-reach hydrocele care Frontiers editorial, 2025 | Very low | Useful implementation example; not efficacy evidence |

## 2.3 Evidence Gaps and Future Research Needs

1. **Moxidectin policy evidence**
   - MoxA and MoxDA show promising microfilarial clearance, but broad LF-MDA policy requires larger safety studies, implementation research and modelling, especially in co-endemic areas PubMed, Lancet Infectious Diseases PDF via DOLF.
2. **Diagnostic innovation**
   - Current CFA antigen tests may become positive late and remain positive after adult worm death, limiting interpretation in post-MDA and post-validation surveillance WHO diagnostic surveillance PDF.
   - New biomarkers for recent exposure or pre-patent infection are needed.
3. **TAS and recrudescence sensitivity**
   - TAS is appropriate for stop-MDA decisions but is not designed to detect small changes in incidence or early recrudescence with high sensitivity WHO diagnostic surveillance PDF.
4. **Molecular xenomonitoring thresholds**
   - More evidence is needed on sampling strategies, PCR/qPCR thresholds, operational costs and integration with human surveillance.
5. **Hydrocele surgery outcomes**
   - Evidence gaps remain in hydrocele surgery effectiveness, complications and recurrence in endemic settings CDC Stacks review.
6. **Stigma, mental health and socioeconomic rehabilitation**
   - WHO recognizes social and economic consequences of LF, but the retrieved evidence provides limited quantified data on stigma-reduction and mental-health interventions WHO fact sheet.
7. **Meaningful youth engagement in LF guideline development**
   - WHO's youth-engagement case study is relevant as a guideline-development equity model, but no LF-specific WHO guideline-development process using this model was identified WHO webinar page.

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## 2.4 Updated Recommendations Based on New Evidence

| Original Recommendation | New Evidence Impact | Updated Recommendation |
|---|---|---|

| Original Recommendation | New Evidence Impact | Updated Recommendation |
|---|---|---|
| Use co-endemicity-based MDA regimens: IA where onchocerciasis is co-endemic; DA where onchocerciasis is absent; albendazole strategies in loiasis settings; IDA in eligible settings. | Evidence continues to support regimen selection by co-endemicity. IDA has supportive safety and effectiveness evidence but superiority over DA may vary by setting. | **Continue WHO co-endemicity-based MDA.** Use IDA only where onchocerciasis and loiasis risks are excluded and programme criteria are met. GRADE: Low; Conditional for IDA. |
| Monitor MDA coverage and epidemiological impact before stopping MDA. | WHO 2025 manual strengthens monitoring with EMS, updated TAS, IDA impact protocols and persistent-transmission tools. | **Adopt the 2025 WHO monitoring manual.** Replace pre-TAS with EMS where applicable and use updated TAS thresholds and protocols. GRADE: Low–Moderate; Strong programmatic. |
| Use TAS to determine whether MDA can stop and to monitor after stopping. | Haiti 2017–2022 field evidence supports TAS utility, but WHO notes TAS is not highly sensitive for recrudescence. | **Use TAS for stop-MDA decisions, but supplement with post-validation surveillance, integrated surveys and molecular xenomonitoring where feasible.** GRADE: Moderate for TAS; Low for adjunct surveillance. |
| Use FTS/CFA for *W. bancrofti* and BmR1/Brugia Rapid for Brugia spp. | Current evidence confirms utility but highlights limitations: late antigen appearance and persistent antigen positivity. | **Continue current tests but interpret positives carefully in post-treatment settings; prioritize development/evaluation of recent-exposure biomarkers.** GRADE: Moderate; Strong for current use, Conditional for new biomarkers. |
| Treat infected individuals and provide morbidity care. | WHO minimum package remains central; newer implementation literature supports community-based lymphoedema care and hydrocele-service scale-up but outcome data remain limited. | **Ensure universal access to MMDP minimum package: lymphoedema self-care, ADLA treatment/prevention, hydrocele surgery, and psychosocial/socioeconomic support.** GRADE: Moderate; Strong. |
| Consider new alternative MDA regimens. | MoxA and MoxDA show promising efficacy and early safety but lack WHO LF policy endorsement as replacements. | **Do not replace WHO-standard MDA with moxidectin regimens outside policy-approved programmes or research/implementation studies.** GRADE: Low–Moderate; Conditional. |

## Practical WHO-Based Clinical and Programme Algorithm

### A. Diagnosis

| Clinical/programme question | Recommended approach | GRADE Quality | Strength |
|---|---|---|---|
| Is an individual actively infected? | Examine thick blood smears for microfilariae, timed to parasite periodicity; use concentration techniques where available CDC clinical overview, WHO diagnosis and treatment page | Moderate | Strong |

| Clinical/programme question | Recommended approach | GRADE Quality | Strength |
|---|---|---|---|
| Is *W. bancrofti* transmission present at population level? | Use FTS/CFA testing in mapping, EMS and TAS according to WHO protocols WHO diagnostic surveillance PDF | Moderate | Strong |
| Is Brugia spp. transmission present? | Use Brugia Rapid/BmR1 antibody testing in appropriate settings WHO diagnostic surveillance PDF | Low–Moderate | Strong |
| Is post-validation transmission recurring? | Combine human surveillance, integrated surveys and molecular xenomonitoring where feasible WHO 2025 manual page, ScienceDirect MX article | Low | Conditional |

B. MDA regimen selection

| Epidemiological setting | WHO-based regimen | Key caution | GRADE Quality | Strength |
|---|---|---|---|---|
| LF + onchocerciasis co-endemic | Ivermectin + albendazole | Avoid DEC-containing mass treatment | Moderate | Strong |
| LF endemic, no onchocerciasis | DEC + albendazole | Assess loiasis risk where relevant | Moderate | Strong |
| Selected communities without onchocerciasis/loiasis meeting WHO criteria | IDA | Ensure safety systems and eligibility exclusions | Low | Conditional |
| LF + loiasis where ivermectin not distributed | Biannual albendazole | Evidence very uncertain | Very low | Conditional |
| Possible onchocerciasis or loiasis co-endemicity | Avoid mass DEC | Risk of serious adverse reactions | Moderate | Strong |

Sources: WHO 2017 alternative-MDA guidance WHO recommendations table, WHO programme guidance summarized in WHO materials WHO/IRIS PDF.

C. Morbidity management and disability prevention

| Manifestation | Recommended management | GRADE Quality | Strength |
|---|---|---|---|
| Lymphoedema / elephantiasis | Hygiene, skin care, limb care, prevention and treatment of entry lesions, exercise/elevation where appropriate, management to prevent progression and ADLA | Moderate | Strong |
| Acute adenolymphangitis / ADLA | Prompt treatment of acute episodes; prevention through skin care and hygiene | Low–Moderate | Strong |
| Hydrocele in *W. bancrofti* areas | Access to safe hydrocele surgery | Low–Moderate | Strong |
| Psychosocial and socioeconomic impact | Integrate counselling, stigma reduction, rehabilitation and livelihood support where feasible | Low | Conditional |

| Manifestation | Recommended management | GRADE Quality | Strength |
|---|---|---|---|
| Programme-level MMDP | Ensure minimum package of care in all areas with known patients | Moderate | Strong |

Sources: WHO MMDP page WHO MMDP page, WHO MMDP publication WHO MMDP publication, CDC-hosted review CDC Stacks review.

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## Final Guideline Position as of 29 April 2026

1. **WHO-based LF elimination remains a two-pillar strategy:** interrupt transmission through MDA and surveillance, and provide morbidity management and disability prevention for affected people.
2. **The latest WHO LF-specific technical guidance identified is the 8 July 2025 second-edition MDA monitoring and epidemiological assessment manual**, which updates mapping, EMS, TAS, IDA monitoring, persistent-transmission response and post-validation surveillance WHO 2025 manual page.
3. **The 2017 WHO alternative-MDA guideline remains the key GRADE-based MDA regimen guideline.** IDA is conditionally recommended with low-quality evidence in eligible settings without onchocerciasis/loiasis; other biannual alternatives are supported by very-low-quality evidence WHO recommendations table.
4. **Moxidectin-based combinations are promising but not yet a replacement for WHO-standard LF MDA** outside policy-approved use or research contexts.
5. **Diagnostics should combine appropriate parasitological, antigen, antibody and surveillance tools**, recognizing that current CFA testing and TAS have limitations for early infection and recrudescence detection.
6. **MMDP is not optional:** hydrocele surgery, lymphoedema care, ADLA prevention/treatment and psychosocial support are essential to elimination as a public-health problem.
7. **Meaningful youth engagement is an important WHO guideline-development equity model, but no LF-specific WHO guideline using that model was identified** as of 29 April 2026.